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Swiss CDISC Submission Guide 2026

A Practical Guide for Sponsors and Project Managers Preparing Regulatory Submissions to Swissmedic

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1. Executive Summary

This guide is written for pharmaceutical and biotechnology sponsors, regulatory affairs leads, and project managers overseeing submissions to Swissmedic. It translates CDISC technical standards into strategic and operational language — helping you understand what is required, why it matters to your timeline, and what Switzerland-specific factors distinguish a Swissmedic submission from an FDA or EMA filing.

It does not replace the technical implementation guides used by your SAS programming or biostatistics teams. Those teams should refer directly to SDTMIG v3.4, ADaM IG v2.1, and the Pinnacle 21 validation ruleset. This document focuses on the decisions, risks, and timelines that senior stakeholders need to manage.

1.1 What You Need to Know — At a Glance

Five things every sponsor should know about CDISC for Swissmedic

- Swissmedic accepts and encourages CDISC-formatted submissions, particularly when aligned with FDA/EMA packages.
- Switzerland has four specific requirements beyond standard CDISC: Swiss trial identifiers, extended metadata, multilingual data, and PASS study traceability.
- The current submission standards are SDTMIG v3.4 (data tabulation) and ADaM IG v2.1 (analysis datasets). These are the implementation guides — the practical rulebooks your programming team follows.
- A Refuse-to-File (RTF) decision from Swissmedic typically delays approval by 3–6 months. The most common causes are incomplete or non-validated CDISC datasets — all preventable.
- CDISC implementation should begin at protocol design, not at database lock. Late engagement is the single most common and costly sponsor mistake.

1.2 Key Risk Summary

Risk	Likelihood	Business Impact	Prevention
Incomplete CDISC domains	High	RTF — 3 to 6 month delay	Automated Pinnacle 21 checks
Controlled terminology violations	High	RTF — submission rejected	Pre-submission CT validation
Missing data traceability	Medium	Extended review queries	SDTM → ADaM → TLF lineage docs
Metadata mismatches in define.xml	Medium	Technical rejection	Define.xml alignment verification
Estimand gaps (ICH E9 R1)	Medium	Efficacy data queries	Protocol-level estimand planning
Swiss-specific ID missing	Medium	Administrative delay	EKOS / cantonal ID integration

1.3 Recommended Actions for Sponsors

1. Confirm your programming team is working to SDTMIG v3.4 and ADaM IG v2.1 — the current regulatory standards.

2. Require Pinnacle 21 validation with zero critical errors before any submission package is assembled.
3. Establish a traceability matrix connecting raw CRF data through SDTM, ADaM, and into the final tables, listings, and figures (TLFs).
4. Document estimands per ICH E9(R1) at protocol stage for pivotal trials — not retrospectively.
5. Engage your CDISC implementation partner at CRF design stage.
6. For Swiss-site studies: plan for multilingual CRF collection and coding dictionaries from the outset.

2. The Regulatory Landscape

2.1 Global Harmonization — What It Means for Sponsors

The period from 2020 to 2026 has seen significant convergence in how regulatory agencies expect clinical data to be structured and submitted. FDA, EMA, and Swissmedic now accept — and increasingly expect — the same CDISC-formatted data packages. A parallel submission strategy, where the same core data package is filed simultaneously at all three agencies, can reduce total submission preparation time by 30–40% compared to building three separate packages.

2.2 FDA Standards Reference (2024–2026)

Standard	Version	Purpose
SDTM Implementation Guide	SDTMIG v3.4	How to structure clinical trial data for submission
ADaM Implementation Guide	ADaM IG v2.1	How to structure analysis datasets
CDISC Controlled Terminology	Latest published	Standardized code lists referenced in define.xml
define.xml	v2.1	Metadata file describing all datasets and variables

Note on terminology: SDTMIG v3.4 is the implementation guide — the practical rulebook built on the SDTM model. Ensure vendor contracts and SOPs reference the implementation guide version, not just the model version.

2.3 Swissmedic Requirements

Swissmedic's adoption of CDISC has followed a steady progression:

Year	Milestone
2016	Swissmedic formally announces acceptance of CDISC-compliant submissions
2019	CDISC compliance referenced in Clinical Trials Ordinance guidance
2022	CPT revision aligns Swiss framework with EU Clinical Trial Regulation
2024	Enhanced PASS study data requirements issued, requiring deeper CDISC traceability

2.4 What Makes Switzerland Different — Four Areas of Specific Attention

Swiss Trial Identification

Every Swiss clinical trial is registered in the Swiss National Clinical Trials Portal (SNCTP) and approved through EKOS. These canton-specific identifiers must be embedded in your CDISC study metadata — in the DM domain, TS domain, and define.xml. Missing or incorrect Swiss identifiers are a common source of administrative delays.

Extended Metadata Requirements

Swissmedic expects metadata that goes beyond standard CDISC define.xml, including additional trial parameter documentation in the TS domain and explicit linkage between the Swiss ethics approval record and the submitted datasets.

Multilingual Data Requirements

Patient-reported outcomes must be validated in local languages. Adverse event verbatim terms from Swiss sites may be in German, French, or Italian — these must be captured as-collected before MedDRA coding. EDC setup must support local language entry from the outset.

Post-Authorization Safety Study (PASS) Data

PASS studies require enhanced CDISC traceability. Swissmedic expects complete lineage from real-world data sources through CDISC-structured datasets to the final safety analysis. See Section 7 for detail.

2.5 The eCTD Submission Architecture

eCTD Module	Content	CDISC Relevance
Module 1	Swiss-specific administrative information	Swiss trial identifiers referenced here
Module 2	Clinical, nonclinical, and quality summaries	Summary statistics drawn from ADaM outputs
Module 5 (5.3.5)	Individual clinical study reports	SDTM datasets, ADaM datasets, define.xml, annotated CRFs, SDRG, ADRG

2.6 ICH E9(R1) Estimands — What Sponsors Need to Know

ICH E9(R1) requires pivotal trials to explicitly define the treatment effect being estimated, including how intercurrent events (treatment discontinuation, rescue therapy, death) are handled in the analysis. There are five recognized approaches:

Approach	Plain language meaning	Typical use
Treatment policy	Measure the effect regardless of whether patients stuck to the treatment	Standard intent-to-treat analyses
Hypothetical	Estimate what the effect would have been if the intercurrent event had not occurred	Discontinuation due to tolerability
Composite	Count the intercurrent event itself as part of the outcome	Death or hospitalization in CV trials

Approach	Plain language meaning	Typical use
While on treatment	Measure the effect only during the period of active treatment	Safety-driven discontinuation
Principal stratum	Measure the effect only in patients who would not experience the intercurrent event	Rescue medication analyses

Estimand decisions drive CRF design and SAP structure. They cannot be retrofitted after database lock without significant regulatory risk. Define them at protocol stage.

3. CDISC Standards — A Sponsor-Level Overview

This section explains what the CDISC standards are and how they relate to each other — without programming detail. Understanding the framework helps sponsors have more productive conversations with CROs and internal programming teams.

3.1 How the Standards Relate to Each Other

Standard	What it does	Analogy
SDTM (via SDTMIG v3.4)	Organizes raw clinical trial data into standardized domains — one per data type	The organized filing system that stores your trial data
ADaM (via ADaM IG v2.1)	Transforms SDTM data into analysis-ready datasets. Every number in your CSR traces back to an ADaM dataset.	The workbench where data is prepared for statistical analysis
define.xml	A metadata file describing every dataset, variable, and code list — the index regulators use to navigate your package.	Table of contents and data dictionary for the whole submission
CDISC Controlled Terminology	Standardized code lists for variables like AE severity, race, route of administration.	The approved vocabulary so terms mean the same thing across agencies

3.2 SDTM Domains — What Sponsors Should Know

Domain	Contains	Swiss-specific consideration
DM — Demographics	One record per subject: age, sex, race, site, treatment	Must include EKOS ethics committee ID and Swiss SNCTP identifier
AE — Adverse Events	All adverse events: term, severity, causality, outcome	Verbatim terms from Swiss sites may be multilingual — capture before MedDRA coding
LB — Laboratory	Lab test results: test, value, units, reference range, baseline	Swiss reference labs may use local conventions — standardization required
EX — Exposure	Actual drug administration: treatment, dose, start/end dates	Critical for PASS — real-world dosing patterns require special handling
DS — Disposition	Study completion and discontinuation records	Lost-to-follow-up tracking important for Swiss PASS studies

3.3 ADaM Datasets — What They Are and Why They Matter

Dataset	Purpose	Key PM concern
ADSL — Subject Level	One row per subject with all population flags and treatment assignments	Population definitions must be locked in the SAP before ADSL is finalized
ADAE — Adverse Events	Analysis-ready AE data with all analysis flags applied	Safety review requires sign-off before submission — allow time
ADLB — Laboratory	Analysis-ready lab data with baseline derivations	Reference range sources must be documented
ADTTE — Time to Event	Survival analysis datasets (OS, PFS, etc.)	Censoring rules must be pre-specified in the SAP

3.4 The Traceability Requirement

One of the most common causes of Swissmedic queries is poor traceability — the inability to follow a number in a summary table back through the analysis dataset to the original CRF data. CDISC requires a complete chain:

Raw CRF Data → **SDTM Datasets** → **ADaM Datasets** → **TLFs in CSR**

This lineage is documented in the Study Data Reviewer's Guide (SDRG) for SDTM and the Analysis Data Reviewer's Guide (ADRG) for ADaM. Both must be submitted. Gaps in traceability are a top-five RTF cause.

4. Refuse-to-File Risk Management

4.1 The Business Cost of an RTF Decision

A Refuse-to-File (RTF) decision means the agency declines to review your submission until identified deficiencies are corrected. The business impact is direct:

Impact	Typical magnitude
Resubmission cycle duration	3 to 6 months from RTF notification to accepted resubmission
Lost revenue during delay (indicative)	For a product with CHF 50M annual revenue, a 4-month delay represents approximately CHF 17M in delayed revenue recognition
Additional remediation costs	Emergency SAS programming, QC, and regulatory management: typically CHF 50K–150K unplanned
Reputational impact	RTF decisions are visible to agency reviewers and may affect perception in future filings

All of the most common RTF triggers are preventable through structured pre-submission validation — making upstream CDISC quality the most cost-effective risk mitigation available.

4.2 The Most Common RTF Triggers — and How to Prevent Them

RTF Trigger	Why it happens	Prevention
Incomplete CDISC domains	Required domains or variables missing	Pinnacle 21: zero Critical and Error before submission
Controlled terminology violations	Non-standard code values used	Lock CT package version at study start; validate pre-submission
Metadata mismatches (define.xml)	Variable names, lengths, or labels differ from actual datasets	Automated define.xml generation and reconciliation against .xpt files
Missing traceability documentation	No SDRG or ADRG, or documents do not match derivations	Author SDRG and ADRG in parallel with dataset development
Estimand deficiencies	ICH E9(R1) framework not followed or not in SAP	Estimands defined at protocol stage; SAP reviewer checks before lock
Missing Swiss identifiers	EKOS or SNCTP absent from metadata	Swiss ID checklist completed during package build

4.3 The Pre-Submission Validation Process

Pinnacle 21 is the industry-standard validation tool for CDISC compliance. It is used by FDA, EMA, and Swissmedic reviewers — meaning the same tool that flags issues at your end will flag them at the agency end.

Severity Level	Meaning	Required action
Critical	Fundamental structural requirement not met — submission will likely be refused	Must be fixed. No exceptions.

Severity Level	Meaning	Required action
Error	Significant non-conformance with CDISC standards	Must be fixed before submission.
Warning	Deviation from standards that may be acceptable with written justification	Justify in SDRG/ADRG or fix.
Note	Informational — unusual value or pattern	Review and document; no fix required.

4.4 Submission Readiness Checklist

30 Days Before Submission

- All required SDTM domains complete and Pinnacle 21 validated (zero Critical and Error findings)
- All ADaM analysis datasets complete and independently QC-reviewed
- Define.xml generated, validated, and reconciled against actual datasets
- Traceability documented: SDRG and ADRG drafted and under review
- Swiss-specific metadata verified: EKOS ID, SNCTP number, cantonal site codes

7 Days Before Submission

- Final Pinnacle 21 validation run: zero Critical and Error findings confirmed
- Programmer, QC, biostatistician, medical writer, and regulatory sign-offs completed

Submission Day

- Final Pinnacle 21 run on assembled package
- eCTD package assembled and integrity-checked
- Upload to Swissmedic portal with confirmation receipt obtained

5. Swissmedic-Specific Requirements

5.1 Swiss Regulatory Structure

Swissmedic is Switzerland's national regulatory authority for therapeutic products. Unlike EMA — which acts as a centralized EU agency — Swissmedic conducts its own independent reviews. In practice, Swissmedic aligns closely with EMA scientific assessments, and sponsors who have EMA approval often pursue Swissmedic approval using a largely common data package.

5.2 Swiss Identification Requirements in Detail

SNCTP — Swiss National Clinical Trials Portal

All trials in Switzerland must be registered in the SNCTP. The registration number must appear in the TS domain, DM domain documentation, and define.xml study metadata.

EKOS — Ethics Committee Online System

Trials require ethics committee approval through EKOS, with canton-specific committees. EC approval dates must be in supplemental regulatory domains; cantonal site codes must be reflected in the DM domain SITEID variable.

5.3 Multilingual Data — Practical Guidance for Sponsors

Data Type	Multilingual requirement	Planning action required
Patient-reported outcomes	Must be administered and collected in patient's local language. Validated translations required.	Confirm validated DE/FR/IT translations exist at protocol design stage
AE verbatim terms	Terms at Swiss sites may be in German, French, or Italian. Must be captured as-reported before MedDRA coding.	EDC must support local language entry. Coding workflow must handle multilingual source terms.
CRF data entry	Swiss site staff may prefer local-language CRF interfaces	Confirm multilingual interface support with EDC vendor. Increases UAT scope.
MedDRA / WHODrug coding	Coding performed in English; a local-language-to-English translation step required before coding	Build translation step into the coding workflow; document in the Data Management Plan

5.4 Parallel Submission Strategy

Agency	Core CDISC standard	Swiss-specific additions
FDA	SDTMIG v3.4 + ADaM IG v2.1	Not applicable
EMA	SDTMIG v3.4 + ADaM IG v2.1	Not applicable
Swissmedic	SDTMIG v3.4 + ADaM IG v2.1	Swiss identifiers (SNCTP, EKOS), multilingual data, PASS traceability, extended TS metadata

The marginal cost of building Swiss-specific elements is typically 5–10% additional effort if planned from the start. Retrofitting after the core package is built costs 2–3x more.

6. Timeline and Resource Guidance

6.1 When to Start CDISC Planning

The most expensive CDISC mistake a sponsor can make

- Waiting until database lock — or later — to engage CDISC implementation resources.
- CRF design decisions made at protocol development directly determine CDISC mapping complexity. A CRF field that does not map cleanly to a CDISC domain requires either a custom domain or laborious post-hoc mapping.
- Rule of thumb: every hour of CDISC planning at protocol stage saves three to five hours of remediation post-lock.

6.2 Phase-Appropriate CDISC Engagement

Trial Phase	CDISC scope	Recommended engagement point
Phase I	Full SDTM for safety and PK; streamlined ADaM for dose escalation and PK analysis	At CRF design — ensure PK and dose escalation data map cleanly to SDTM
Phase II	Full SDTM; standard ADaM for dose-response and preliminary efficacy	At protocol design — estimand strategy and population flags need pre-specification
Phase III (Pivotal)	Full CDISC compliance: all domains, complete ADaM, define.xml, SDRG, ADRG	At study design — CDISC specifications should be in the CRO selection RFP
PASS / RWE	SDTM with real-world data extensions; enhanced Swissmedic PASS traceability	At study protocol — RWD mapping to CDISC is non-trivial; engage early

6.3 Indicative Timelines for CDISC Implementation

Activity	Clean (CDISC from design)	Retrofit (post-lock)
CRF-to-SDTM specifications	2–4 weeks	4–10 weeks
SDTM dataset programming and QC	4–8 weeks	8–16 weeks
ADaM dataset programming and QC	4–8 weeks	6–12 weeks
Define.xml generation and validation	1–2 weeks	2–4 weeks
SDRG and ADRG authoring	1–2 weeks	2–3 weeks
Pinnacle 21 validation and remediation	1–2 weeks	3–6 weeks
Total (indicative)	13–26 weeks	25–51 weeks

Ranges assume a Phase III study of moderate complexity (10–15 domains, 3–4 key ADaM datasets). Very large safety databases or studies with extensive RWD integration require additional time.

6.4 Common Mistakes That Drive Cost and Delay

Mistake	How it happens	Typical cost impact
Partial CDISC conversion	Sponsor converts pivotal study only; agency requests all studies	Full re-engagement of programming team; 8–16 weeks additional work
Inconsistent MedDRA dictionary version	Different studies coded to different MedDRA versions	Recoding exercise across affected studies; delays safety database lock
Incomplete define.xml	ValueListDef not completed for key variables	Define.xml rework; typically 2–4 weeks
Outdated controlled terminology	CT package used at programming differs from what define.xml references	CT remediation across all datasets; Pinnacle 21 rerun cycle
Late Swiss identifier integration	EKOS and SNCTP IDs added late in package assembly	Metadata rework in DM, TS, and define.xml; submission delay risk

7. PASS and Real-World Evidence Studies

7.1 PASS Study Context for Swissmedic

Post-Authorization Safety Studies (PASS) are frequently a condition of Swissmedic approvals, particularly for products approved under conditional pathways. The 2024 update to Swissmedic's PASS guidance introduced enhanced CDISC traceability requirements — the standard of data organization expected for interventional trials now applies to PASS studies.

7.2 Real-World Data Sources and CDISC Mapping

RWD Source	CDISC domains used	Key mapping challenge
Swiss health insurance claims	Healthcare Economics and Utilization domains (HO, HU)	ICD-10 codes require mapping to MedDRA and standardized CDISC terminology
Hospital electronic records	Multiple domains depending on data collected (AE, LB, VS, CM)	Data quality and completeness vary; imputation rules must be pre-specified
Swiss patient registries	SDTM with custom domain extensions where needed	Longitudinal patient linkage across sources requires careful subject ID management
Spontaneous AE reports	AE domain with SADR flagging	Duplicate detection and causality assessment are additional steps vs. interventional SDTM

7.3 The Date Imputation Challenge in RWD

Real-world data frequently has partial or missing dates — a hospital record may show month and year but not day. CDISC requires ISO 8601 date format with specific handling for partial dates. Imputation rules must be pre-specified in the PASS protocol and data management plan — they are scrutinized by Swissmedic reviewers because imputation choices can materially affect time-to-event analyses. This is a planning issue: the rules must be established before data access.

7.4 PASS CDISC Checklist for Sponsors

Planning area	Required action	Timing
Data source access agreements	Confirm CDISC-compatible data extraction is possible from each source	Before protocol finalization
Date imputation rules	Pre-specify in protocol and DMP; reviewed by biostatistician and regulatory lead	Before protocol finalization
ICD-to-MedDRA mapping	Establish mapping table for all expected diagnoses; plan for unmappable codes	At protocol stage
Swiss identifier integration	EKOS approval, SNCTP registration, and cantonal codes documented	At study setup

Planning area	Required action	Timing
SADR flagging strategy	Define criteria for suspected adverse drug reactions in the PASS context	At protocol stage
Enhanced traceability	ADRG to include full lineage from RWD source through SDTM to analysis	At data collection, not at analysis

8. How Datametrix Can Help

8.1 Our Role

Datametrix AG is a CDISC Silver Partner based in Neuchâtel, Switzerland, specializing in clinical data management, biostatistics, and SAS programming for regulatory submissions — with particular expertise in Swissmedic filings. Our multilingual team (German, French, Italian, English) has supported Swissmedic submissions since 2003.

We work in three modes: as primary CDISC implementation partner from protocol design through submission; as QC and validation partner reviewing an existing team's work; or as emergency remediation support following an RTF or regulatory query.

8.2 Service Areas

Service	What it covers
CDISC Mapping and Specification	CRF-to-SDTM and ADaM dataset specifications completed before programming begins
SDTM and ADaM Programming	SAS-based metadata-driven programming using Pinnacle 21-validated workflows
Pinnacle 21 Validation	Pre-submission validation with full issue triage, remediation, and justified deviation documentation
Define.xml and Metadata	Complete generation, value-level metadata, and alignment verification
SDRG and ADRG Authoring	Reviewer's guide preparation for both SDTM and ADaM packages
Swissmedic Submission Support	Swiss identifier integration, multilingual data, EKOS coordination, eCTD Module 5 assembly
PASS and RWE Implementation	RWD CDISC mapping, ICD-to-MedDRA mapping, date imputation specification, enhanced traceability
CDISC Training Workshops	Half-day and full-day workshops: CDISC fundamentals, Swiss-specific requirements, estimand strategy

8.3 Free CDISC Readiness Assessment

We offer a complimentary 30-minute CDISC Readiness Assessment for sponsors preparing a Swissmedic submission. We cover: CDISC maturity gap analysis, dataset validation status, Swiss-specific compliance review, indicative project timeline, and ranked RTF risk list.

9. Glossary of Key Terms

Defines the technical terms used in this guide for sponsors and project managers who work with CDISC submissions.

Term	Definition
ADaM	Analysis Data Model. The CDISC standard for analysis-ready datasets. ADaM IG v2.1 is the current implementation guide.
ADRG	Analysis Data Reviewer's Guide. Submitted alongside ADaM datasets, explaining derivations and how to navigate the analysis package.
Annotated CRF	A copy of the Case Report Form with annotations showing how each field maps to a specific SDTM domain and variable.
CDISC	Clinical Data Interchange Standards Consortium. The global standards body that defines SDTM, ADaM, and related standards.
Controlled Terminology (CT)	Standardized code lists defining acceptable values for specific CDISC variables (e.g., adverse event severity). Must use the version current at time of submission.
CPT	Clinical Trials Ordinance. Swiss regulation governing clinical trial conduct. Revised in 2022 to align with EU CTR.
Define.xml	An XML metadata file submitted with every CDISC package. Describes every dataset, variable, code list, and derivation — the index regulators use to navigate the data.
eCTD	Electronic Common Technical Document. The standard electronic format for regulatory submissions to FDA, EMA, and Swissmedic. CDISC datasets sit within Module 5.
EKOS	Ethics Committee Online System. The Swiss system for ethics committee submissions and approvals. Canton-specific.
Estimand	A precise specification of the treatment effect being estimated, including how intercurrent events are handled. Required by ICH E9(R1).
ICH E9(R1)	International Council for Harmonisation guideline on statistical principles for clinical trials — with addendum on estimands. Published 2019.
MedDRA	Medical Dictionary for Regulatory Activities. International medical coding terminology for adverse events and diagnoses.
PASS	Post-Authorization Safety Study. An observational study required as a condition of authorization to collect additional real-world safety data.
Pinnacle 21	Industry-standard software for CDISC compliance validation. Produces critical / error / warning / note findings. Used by sponsors, CROs, and regulators.
RTF	Refuse-to-File. Agency decision to decline review of a submission due to fundamental deficiencies. Typically causes a 3–6 month delay.
RWD / RWE	Real-World Data / Real-World Evidence. Data from outside controlled trials (hospital records, insurance claims, registries) and the evidence derived from it.
SAP	Statistical Analysis Plan. Pre-specified document describing all statistical analyses. Must be finalized before database lock.

Term	Definition
SDTM	Study Data Tabulation Model. CDISC standard for organizing raw clinical data into submission-ready datasets. SDTMIG v3.4 is the current implementation guide.
SDRG	Study Data Reviewer's Guide. Submitted alongside SDTM datasets, explaining study design, data collection, and dataset structure.
SNCTP	Swiss National Clinical Trials Portal. Official Swiss registry for clinical trials. Registration is mandatory for all trials in Switzerland.
TLF	Tables, Listings, and Figures. The outputs in a Clinical Study Report generated from ADaM analysis datasets.
TPA	Therapeutic Products Act. The primary Swiss federal law governing medicinal products and medical devices.
ValueListDef	A component of define.xml providing metadata at the value level. Commonly missing in incomplete submissions.
.xpt file	SAS transport file format. All CDISC datasets are submitted in this format. Has specific technical constraints (8-character variable names, 200-character labels).

10. References and Further Reading

Reference	Source
SDTM Implementation Guide v3.4	CDISC. www.cdisc.org/standards/foundational/sdtmig
ADaM Implementation Guide v2.1	CDISC. www.cdisc.org/standards/foundational/adam
Study Data Technical Conformance Guide (March 2024)	U.S. Food and Drug Administration. www.fda.gov
ICH E9(R1) — Estimands and Sensitivity Analysis	International Council for Harmonisation. 2019. www.ich.org
Swissmedic Clinical Trials Guidance	Swissmedic. www.swissmedic.ch
Clinical Trials Ordinance (CPT) 2022 Revision	Swiss Federal Council. www.fedlex.admin.ch
eCTD Specification v3.2.2	International Council for Harmonisation. 2021. www.ich.org
DARWIN EU RWD Specifications	European Medicines Agency. 2024. www.ema.europa.eu

Regulatory Disclaimer

This guide reflects Datametrix interpretation of publicly available regulatory guidance from Swissmedic, FDA, EMA, and CDISC. Sponsors should verify specific requirements directly with regulatory authorities. CDISC standards evolve; consult current implementation guides for authoritative specifications. This document does not constitute legal or regulatory advice.

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